Advanced ALK-positive non-small-cell lung cancer (NSCLC) after crizotinib Adults with ALK-positive advanced non-small-cell lung cancer previously treated with crizotinib: National Cancer Control Programme (NCCP) 00340a. This suite does not turn the broader first-line licence into an NCCP indication or funding approval. Confirm your treatment setting with the team.

NCCP 00340 · VERSION 4

Ceritinib monotherapy

The usual starting dose is 450 mg once daily with food, every day without a planned rest week. Follow your own prescription. Treatment continues while it helps and side effects remain acceptable; there is no fixed course length.

Make treatment fit into daily life

Take at the same time daily with food. Swallow capsules whole with water; do not chew or crush. Take a missed dose unless the next is due within 12 hours; ask your pharmacist if timing is unclear. Never double a dose. If you vomit, do not replace the dose; take the next at its usual time. If illness prevents you eating with the dose, contact the team. Only the prescriber can arrange a different fasting dose. Do not alternate between fed and fasting schedules or change the dose yourself.

Before starting

Confirm ALK result, previous treatment, prescription, baseline tests, supply and contact numbers.

Taking treatment

Take at the same time daily with food. Swallow capsules whole with water; do not chew or crush. Take a missed dose unless the next is due within 12 hours; ask your pharmacist if timing is unclear. Never double a dose. If you vomit, do not replace the dose; take the next at its usual time.

Food or illness

If illness prevents you eating with the dose, contact the team. Only the prescriber can arrange a different fasting dose. Do not alternate between fed and fasting schedules or change the dose yourself.

Planned checks

Liver tests every 2 weeks for 3 months, then monthly. Blood count, kidneys and fasting glucose every 4 weeks; electrocardiogram (ECG) at 1 month, then as needed. Extra checks depend on symptoms.

Refills and interruptions

Request repeats early. Keep an up-to-date medicine list. Ask for written dose and food instructions after any change; only the prescriber authorises restart.

Who arranges each check?

Agree the due window, booking team, result reviewer and whom to contact if an appointment or result is missing. This reference page does not book or record a test.

Sources and review

The NCCP regimen supplies the treatment rules. Separate guidelines supply the longer-term monitoring recommendations, with their qualifications retained.

- NCCP 00340 v4 · official regimen
- EMA Zykadia product information, retrieved 29 October 2026
- NCCP systemic anticancer therapy (SACT) Antiemetic Guidance V6 (2025), Tables 5 and 8
- NCCP supportive antiemetics for NCIS Medical Oncology; check local policy
- ASCEND-5: post-chemotherapy/crizotinib phase III trial, Shaw 2017
- ASCEND-8: pharmacokinetic and food-effect study, Cho 2017
- ASCEND-8: primary efficacy and safety analysis, Cho 2019

Source reconciled; independent clinical, pharmacy and patient/PPI review pending